

Comprehensive Synthetic Medical Record Packet

John Alvarez - MRN 14332

Important This is a synthetic test-patient packet created for EHR and OmniScribe workflow testing only. It intentionally contains rich, realistic clinical details but no real patient data. Do not use this packet for patient care, coding, legal release, billing, or clinical decision-making.			
Patient name	John Alvarez	MRN	14332
DOB / age	03/14/1956 / 70 years	Sex	Male
Address	120 Cedar Ridge Way, Nespelem, WA 99155 (synthetic)	Phone	(555) 014-3320
Primary care clinic	Lake Roosevelt Community Health Centers - Demo Clinic	PCP	Alina Martin, MD
Emergency contact	Maria Alvarez, spouse - (555) 019-7412	Preferred language	English
Release packet period	04/18/2025 through 05/29/2026	Record class	Scanned external records plus current outpatient notes

Allergen	Reaction / symptoms	Severity	Clinical instructions
Penicillin	Anaphylaxis with urticaria and throat tightness in early adulthood. Family reports ER treatment.	Severe	Avoid penicillins. Verify before cephalosporins. ID/allergy consult if beta-lactam is unavoidable.
Bee stings / hymenoptera venom	Anaphylaxis with wheeze, facial swelling, hypotension. Last event documented 2019.	Severe	Carries epinephrine auto-injector. Reinforce emergency action plan. Avoid outdoor exposure without kit.
Latex	Contact dermatitis with rash; reported wheezing when exposed to latex gloves during prior procedure.	Moderate to severe	Latex-free precautions for all visits, blood draws, rehab equipment, catheters, and dressings.

Packet generated date: 05/29/2026. Intended upload location: Patient > Upload records in OmniScribe demo environment.

Synthetic source systems represented: primary care EHR, transplant center records, acute hospital discharge records, lab interface, imaging/radiology reports, outpatient PT/OT documentation, and scanned care coordination forms.

Record Index and Packet Contents

Section	Document types included	Clinical emphasis
Administrative and safety	Demographics, release summary, allergies, active problem list, medication list	Penicillin, bee sting, and latex precautions; transplant status; fall risk
Primary care	Transitional care note, chronic care follow-up, medication reconciliation	Hypertension, diabetes, CKD, transplant coordination, CVA residual deficits
Heart transplant hospitalization	Discharge summary, operative report, anesthesia/perfusion summary, ICU notes, step-down notes, consults	Open heart transplantation, ICU course, immunosuppression, post-op complications
Prior CVA hospitalization	Acute hospital discharge summary, neurology progress, therapy/swallow consults, imaging	Right MCA ischemic stroke about one year before current record; residual left-sided deficits
Laboratory reports	CBC, CMP, tacrolimus troughs, endocrine, lipid panel, infection surveillance, urinalysis	Recent values from April-May 2026; abnormal flags and trends
Imaging and diagnostic reports	Brain CT/MRI/CTA, chest imaging, echocardiogram, RHC, endomyocardial biopsy, EKG	Stroke history, transplant surveillance, cardiopulmonary status
Rehabilitation	Outpatient PT evaluation, daily notes, progress note, OT evaluation, daily note, plan of care	Gait, balance, endurance, left upper-extremity coordination, ADL/IADL recovery
Care coordination and coding reference	DME, home health, immunizations, health maintenance, sample ICD/CPT snapshot	Workflow testing data; not a billable claim
Release format note The packet is formatted like a compiled scanned medical-record release. Every page carries a synthetic-record warning. Some sections are intentionally redundant because real medical-record releases often repeat medication lists, allergies, diagnoses, and signatures from different source systems. Dates, names, facilities, results, and signatures are fictitious and internally consistent for test parsing.		

Demographics, Emergency Contacts, Safety Alerts

Patient name	John Alvarez	MRN	14332
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Allergy and precaution banner

Allergen	Reaction / symptoms	Severity	Clinical instructions
Penicillin	Anaphylaxis with urticaria and throat tightness in early adulthood. Family reports ER treatment.	Severe	Avoid penicillins. Verify before cephalosporins. ID/allergy consult if beta-lactam is unavoidable.
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Latex	Contact dermatitis with rash; reported wheezing when exposed to latex gloves during prior procedure.	Moderate to severe	Latex-free precautions for all visits, blood draws, rehab equipment, catheters, and dressings.

Visit-level safety requirements

Risk area	Required action
Latex precautions	Use latex-free gloves, tourniquets, BP cuff components, therapy bands, catheter supplies, wound dressings, and emergency equipment.
Immunosuppression	Screen for fever, cough, wound drainage, diarrhea, oral thrush, medication missed doses, and recent sick contacts at every encounter.
Fall risk	Uses single-point cane; document gait belt use during challenging rehab tasks; supervise treadmill and stair training.
Sternal / transplant recovery precautions	As of May 2026, sternotomy healed; still avoid abrupt heavy lifting until cleared by transplant team; monitor HR response and RPE.
Anaphylaxis preparedness	Confirm epinephrine auto-injector availability and expiration date; bee sting history should be visible in chart banner.

Active Problem List and Current Medication Summary

Problem	Onset / status	Details and monitoring needs
Heart transplant recipient - orthotopic heart transplant	02/07/2026 - active	Post OHT for end-stage ischemic cardiomyopathy. On tacrolimus, mycophenolate, prednisone taper. Surveillance biopsy 05/10/2026: 0R, pAMRO.
Immunosuppression due to drug therapy	Active	Monitor tacrolimus trough, CBC, CMP, infection symptoms, CMV PCR, renal function, and medication interactions.
Hypertension	Longstanding; worsened post transplant	Home BP range 132-158/76-92. Goal documented by transplant clinic: less than 130/80 if tolerated.
History of ischemic CVA - right MCA	04/18/2025	Residual left-sided weakness, slowed fine motor coordination, gait instability, and mild executive attention deficits.
Type 2 diabetes mellitus with steroid-related hyperglycemia	Active	A1c 7.6% on 05/21/2026. Uses basal insulin and mealtime correction scale while prednisone taper continues.
Chronic kidney disease stage 3a	Active	Creatinine 1.42, eGFR 53 on 05/21/2026. Avoid nephrotoxins; tacrolimus level monitored closely.
Hyperlipidemia	Active	LDL 72 on pravastatin; avoid strong CYP3A interactions with tacrolimus.
Postoperative anemia	Improving	Hgb 11.8 on 05/21/2026, previously 9.6 at transplant discharge.
Deconditioning / high fall risk	Active	TUG 18.6 sec at PT evaluation. Uses single-point cane outdoors. Outpatient PT/OT active.
Obstructive sleep apnea	Active	Uses CPAP at home, reports 5-6 hours/night.
Benign prostatic hyperplasia	Active	Stable on tamsulosin; nocturia 1-2 times.

Medication list as reconciled 05/21/2026

Medication	Dose / route / schedule	Indication	Notes
Tacrolimus	2 mg PO every morning and 1.5 mg PO every evening	Transplant immunosuppression	Target trough 8-12 ng/mL. Level 10.2 on 05/21/2026.
Mycophenolate mofetil	1000 mg PO twice daily	Transplant immunosuppression	Monitor CBC and GI tolerance.
Prednisone	10 mg PO daily	Post-transplant steroid taper	Taper per transplant protocol; contributes to hyperglycemia.
Valganciclovir	450 mg PO daily	CMV prophylaxis	CMV D+/R+ documented. Renal dose adjusted.
Trimethoprim-sulfamethoxazole SS	1 tablet PO Mon/Wed/Fri	PJP prophylaxis	No sulfa allergy reported.
Nystatin oral suspension	5 mL swish and swallow four times daily	Fungal prophylaxis	Continue while prednisone dose remains elevated.
Aspirin	81 mg PO daily	Cardiac allograft vasculopathy prophylaxis	Continue unless bleeding.
Pravastatin	20 mg PO nightly	Lipid control / allograft vasculopathy prophylaxis	Chosen for lower interaction risk.
Amlodipine	10 mg PO daily	Hypertension	Increased at PCP visit 05/21/2026.

Medication Reconciliation Continued and Clinical Timeline

Medication	Dose / route / schedule	Indication	Notes
Hydralazine	25 mg PO three times daily if SBP persistently above 150	Hypertension bridge plan	Hold if dizziness or SBP less than 110.
Insulin glargine	14 units SQ nightly	Diabetes / steroid hyperglycemia	Adjust after prednisone taper.
Insulin aspart	Sliding scale with meals	Post-prandial hyperglycemia	Uses home glucose log.
Metformin XR	500 mg PO with evening meal	Type 2 diabetes	Renal function adequate; hold if acute illness or contrast study.
Pantoprazole	40 mg PO daily	GI protection / GERD	Continue during steroid therapy.
Magnesium oxide	400 mg PO twice daily	Tacrolimus-associated magnesium wasting	Mg 1.6 on 05/21/2026.
Calcium carbonate + Vitamin D3	600 mg/800 IU PO twice daily	Bone protection	Steroid exposure.
Tamsulosin	0.4 mg PO nightly	BPH	Monitor orthostasis.
Sertraline	50 mg PO daily	Depression/anxiety symptoms	Started after 2025 stroke; stable mood.
Epinephrine auto-injector	0.3 mg IM as needed for anaphylaxis	Bee sting allergy	Keep two devices available.

Clinical timeline

Date	Facility / setting	Event summary
04/18/2025	Columbia River Acute Hospital	Admitted with acute right MCA ischemic stroke; NIHSS 9; residual left hemiparesis.
04/23/2025	Inpatient rehabilitation transfer	Discharged from acute stroke hospitalization to inpatient rehab; anticoagulation initiated for cardioembolic risk.
2025 - late 2025	Cardiology / HF clinic	Progressive ischemic cardiomyopathy and chronic HFrEF; transplant evaluation completed.
02/03/2026	Inland Northwest Transplant Center	Admitted with cardiogenic shock; IABP support; listed for transplant.
02/07/2026	Cardiothoracic OR	Redo sternotomy and orthotopic heart transplantation.
02/07/2026 - 02/16/2026	Cardiovascular ICU	Ventilator support, inotropes, immunosuppression initiation, AKI, delirium, rehab mobilization.
02/17/2026 - 03/02/2026	Transplant step-down unit	Medication stabilization, biopsy surveillance, discharge teaching, PT/OT clearance.
03/02/2026	Home with home health	Discharged home with spouse support, home nursing, cane, shower chair, glucose/BP monitoring.
04/08/2026	Primary care TCM visit	Medication reconciliation, allergy precautions, BP/DM follow-up, rehab referral.
04/15/2026	Outpatient physical therapy	PT initial evaluation for deconditioning, gait instability, stroke residual deficits.
04/18/2026	Outpatient occupational therapy	OT initial evaluation for left arm coordination and ADL/IADL deficits.
05/10/2026	Transplant clinic / cath lab	RHC and endomyocardial biopsy; normal filling pressures; biopsy 0R/pAMR0.
05/21/2026	Primary care follow-up	Hypertension and diabetes management; recent labs and chest imaging reviewed.

Primary Care Transitional Care Visit - 04/08/2026

Encounter type	Transitional care management after transplant hospitalization	Provider	Alina Martin, MD
Location	Lake Roosevelt Community Health Centers - Primary Care	Status	Signed 04/08/2026 18:12
Chief concern	Post-hospital follow-up after open heart transplant and medication reconciliation	Accompanied by	Spouse Maria Alvarez

Subjective

Patient returns after discharge from transplant center on 03/02/2026. He reports improved dyspnea compared with pre-transplant baseline and is walking in the hallway at home with cane. He denies fever, chills, productive cough, chest pressure, syncope, palpitations, wound drainage, or missed immunosuppression doses. Spouse manages pill box and logs blood pressure, glucose, temperature, and weight twice daily. Home BP mostly 138-152 systolic. Fasting glucose 138-188 with prednisone taper. No bee sting exposure. Latex allergy reviewed with clinic team.

Objective

Vital / exam item	Finding
Vitals	BP 148/88 sitting, 142/84 repeat; HR 96 regular; RR 16; Temp 98.2 F; SpO2 98% RA; Weight 187 lb
General	Alert, cooperative, mildly fatigued, no acute distress.
Cardiovascular	Regular rhythm; no murmur; sternotomy incision well approximated without erythema or drainage.
Pulmonary	Clear to auscultation, no wheeze, no rales.
Neuro / gait	Mild residual left facial flattening; left grip 4-/5, left hip flexion 4/5; gait slow with cane, no foot drop observed.
Skin	No rash; bruising at prior venipuncture sites; no pressure injury.

Assessment and plan

Assessment	Plan
Status post heart transplant	Coordinate with transplant center. Continue tacrolimus/mycophenolate/prednisone/prophylaxis. Reinforced do not stop or delay immunosuppression. Labs weekly per transplant protocol.
Hypertension	Continue amlodipine 5 mg daily at that time; send home BP log to transplant nurse. Avoid ACE/ARB initiation until renal function and tacrolimus levels stable.
Steroid hyperglycemia / T2DM	Continue insulin glargine and sliding scale. Reviewed hypoglycemia precautions. Refer to diabetes education.
CVA residual deficits and deconditioning	Outpatient PT and OT referrals placed. Fall precautions and cane use reviewed.
Allergies	Chart banner updated for penicillin, bee sting, and latex. Epinephrine auto-injector refill sent.

Electronic signature	Electronically signed by Alina Martin, MD	Date/Time	04/08/2026 18:12
Authentication statement	This synthetic note was generated for OmniScribe testing and is not a real clinical record.	Status	Signed

Primary Care Follow-up Visit - 05/21/2026

Encounter type	Established patient chronic care follow-up	Provider	Alina Martin, MD
Chief concerns	Blood pressure, diabetes, transplant follow-up, rehab progress	Status	Signed 05/21/2026 17:42

History of present illness

John Alvarez is a 70-year-old male with orthotopic heart transplant on 02/07/2026, prior right MCA CVA, hypertension, type 2 diabetes, CKD stage 3a, and severe allergies to penicillin, bee stings, and latex. He reports improved stamina with outpatient rehab. He can walk from parking lot to clinic using cane with one standing rest break. No chest pain, orthopnea, PND, fever, sore throat, drainage from incision, or missed transplant medication. BP log remains above goal, commonly 138-156/78-92. Morning glucose 126-172; evening glucose 180-230 when prednisone taken with breakfast. He requests renewal of rehab plan and clarification on whether he can resume light yard activity.

Review of systems

System	Positive / negative elements
Constitutional	Fatigue improving; denies fever, chills, night sweats, unintentional weight loss.
Cardiovascular	Denies chest pressure, palpitations, syncope, new edema. Mild exertional dyspnea with stairs, improving.
Respiratory	Denies cough or wheeze. Uses CPAP.
Neurologic	Stable left arm weakness and reduced fine motor speed; denies new speech difficulty, facial droop, vision loss, or acute weakness.
Endocrine	Hyperglycemia with steroid taper; no severe hypoglycemia.

Objective

Vitals	BP 142/86 left arm latex-free cuff; repeat 136/82; HR 94; RR 16; Temp 98.0 F; SpO2 98% RA; Weight 190 lb; BMI 28.9
Exam	Well appearing. Oropharynx without thrush. Sternotomy healed. Heart regular, no rub. Lungs clear. No leg edema. Left grip 4-/5, left shoulder flexion 4/5, left hip flexion 4/5; gait stable with cane.

Assessment and plan

Diagnosis / problem	Plan
Hypertension	Increase amlodipine to 10 mg daily. Continue home BP log. Hydralazine bridge plan if SBP persistently above 150 per transplant cardiology. Recheck in 2 weeks.
Heart transplant recipient	Continue current immunosuppression. Tacrolimus trough at goal. Follow transplant clinic 06/03/2026. Avoid live vaccines. Call urgently for fever above 100.4 F.
Diabetes / steroid hyperglycemia	Continue glargine 14 units nightly and mealtime correction. Start/continue metformin XR 500 mg with evening meal; hold during acute illness or contrast.
CVA residual deficit / deconditioning	Continue PT/OT 2 times weekly. No heavy yard work yet; begin light supervised walking program, RPE 11-13, follow transplant rehab precautions.
CKD stage 3a	Monitor CMP with transplant labs. Avoid NSAIDs and dehydration.
Allergies	Epinephrine prescription active. Latex-free precautions reinforced. Penicillin allergy visible in medication safety review.

Electronic signature	Electronically signed by Alina Martin, MD	Date/Time	05/21/2026 17:42
Authentication statement	This synthetic note was generated for OmniScribe testing and is not a real clinical record.	Status	Signed

Transplant Cardiology Clinic and Biopsy Visit - 05/10/2026

Facility	Inland Northwest Transplant Center - Heart Transplant Clinic	Provider	Renee Park, MD
Visit type	Post-OHT surveillance, right-heart cath, endomyocardial biopsy	Status	Signed 05/10/2026 16:30
Transplant date	02/07/2026	CMV serostatus	Donor positive / recipient positive

Interval history

Patient is 3 months post orthotopic heart transplantation for ischemic cardiomyopathy. No rejection treatment to date. Reports medication adherence with spouse double-checking pillbox. No fevers, diarrhea, new cough, wound drainage, edema, or orthopnea. Rehab participation increasing; walking 8-10 minutes at a time. Home BP above goal. Blood sugars elevated after breakfast prednisone dose. Tremor mild and worse when tacrolimus level high. Denies severe headache or new neurologic deficits.

Right-heart catheterization and biopsy summary

Measurement / item	Result	Interpretation
RA mean	6 mmHg	Normal right-sided filling pressure
RV	30/6 mmHg	No RV pressure overload
PA	28/12 mmHg, mean 18	Normal pulmonary pressure
PCWP	11 mmHg	Normal left-sided filling pressure
Thermodilution cardiac index	2.6 L/min/m2	Adequate
Fick cardiac index	2.8 L/min/m2	Adequate
Endomyocardial biopsy	ISHLT 0R, pAMR0	No acute cellular or antibody-mediated rejection
Donor-specific antibodies	Negative	No DSA detected 05/10/2026

Plan

Care domain	Plan
Immunosuppression	Continue tacrolimus target trough 8-12, mycophenolate 1000 mg BID, prednisone 10 mg daily; consider prednisone taper to 7.5 mg if next biopsy remains negative.
Infection prophylaxis	Continue valganciclovir and TMP-SMX. CMV PCR every 2 weeks until prophylaxis complete.
Hypertension	Primary care may increase amlodipine; avoid diltiazem/verapamil because of tacrolimus interaction unless transplant team directs.
Rehabilitation	Cleared for structured outpatient cardiac-informed rehab with HR/RPE monitoring. Avoid maximal effort testing until 6 months post transplant.

Electronic signature	Electronically signed by Renee Park, MD	Date/Time	05/10/2026 16:30
Authentication statement	This synthetic note was generated for OmniScribe testing and is not a real clinical record.	Status	Signed

Acute Hospital Discharge Summary - Open Heart Transplant Stay - Page 1

Hospital	Inland Northwest Transplant Center	Admission date	02/03/2026
Discharge date	03/02/2026	Attending	Victor Han, MD - Transplant Cardiology
Admission reason	Cardiogenic shock due to end-stage ischemic cardiomyopathy	Disposition	Home with spouse support, home health nursing, outpatient transplant follow-up

Discharge diagnoses

Primary / secondary diagnosis	Details
Status post orthotopic heart transplantation	Procedure date 02/07/2026; redo sternotomy; explant of diseased recipient heart and ICD system.
Cardiogenic shock - resolved	Required pre-operative intra-aortic balloon pump and inotropic support.
Acute hypoxic respiratory failure - resolved	Postoperative ventilation; extubated POD1 to nasal cannula.
Acute kidney injury on CKD 3a - improved	Creatinine peaked at 1.94 mg/dL, improved to 1.36 mg/dL by discharge.
Postoperative anemia - stable	No active bleeding at discharge; Hgb 9.6 g/dL at discharge.
Steroid-induced hyperglycemia / type 2 diabetes	Discharged on basal insulin and correction scale.
Hypertension after transplant	Controlled with amlodipine at discharge; outpatient titration expected.
Delirium - resolved	Brief ICU delirium POD2-POD4; no ongoing psychosis or safety concern.
History of right MCA stroke	Residual left-sided weakness; inpatient PT/OT involved; outpatient therapy recommended.

Hospital course summary

Patient admitted 02/03/2026 with low-output heart failure, volume overload, worsening renal function, and hypotension despite guideline-directed therapy. Right-heart cath showed cardiogenic shock physiology. Intra-aortic balloon pump was placed and patient was admitted to the cardiovascular ICU. He underwent orthotopic heart transplantation on 02/07/2026. Postoperative graft function was good by TEE. ICU course included vasoactive/inotropic support, mechanical ventilation, AKI, hyperglycemia, and delirium. He was extubated POD1, IABP removed POD2, transferred to step-down POD10, and completed transplant medication teaching prior to discharge.

Acute Hospital Discharge Summary - Open Heart Transplant Stay - Page 2

Procedures and major diagnostics

Date	Procedure / study	Findings / result
02/03/2026	Right-heart catheterization	RA 14, PA 52/28, PCWP 26, CI 1.6; consistent with cardiogenic shock.
02/03/2026	Intra-aortic balloon pump placement	Right femoral IABP placed without documented complication.
02/07/2026	Orthotopic heart transplantation	Redo sternotomy; donor heart implanted; immediate graft function good.
02/08/2026	Extubation	Extubated to 4 L nasal cannula, then room air by POD5.
02/09/2026	IABP removal	Hemodynamically stable after removal.
02/21/2026	Endomyocardial biopsy #1	0R, pAMR0.
02/28/2026	Endomyocardial biopsy #2	1R mild cellular rejection, pAMR0; no pulse steroid required.

Discharge condition and instructions

Domain	Instructions		
Activity	Walk 3-5 times daily as tolerated. No lifting above 10 lb until cleared. Avoid pushing/pulling with arms during early sternotomy recovery. Use cane on uneven surfaces.		
Diet	Heart-healthy, diabetic consistent carbohydrate, low microbial food safety guidance for transplant recipient.		
Wound care	Sternotomy open to air if dry. Call for redness, warmth, drainage, separation, fever, or increasing pain.		
Monitoring	Record temperature, BP, HR, weight, and glucose daily. Call transplant team for fever >100.4 F, weight gain >2 lb/day or >5 lb/week, dyspnea, dizziness, vomiting/diarrhea, or missed immunosuppression.		
Follow-up	Transplant clinic with labs within 72 hours; biopsy schedule per protocol; primary care within 2 weeks; PT/OT after home health transition.		
Allergy precautions	Penicillin, bee stings, and latex listed on discharge medication safety sheet and home health referral.		
Electronic signature	Electronically signed by Victor Han, MD	Date/Time	03/02/2026 12:14
Authentication statement	This synthetic note was generated for OmniScribe testing and is not a real clinical record.	Status	Signed

Transplant Discharge Medication and Follow-up Orders - 03/02/2026

Discharge medication highlights

Class	Medication and order	Monitoring / rationale
Immunosuppression	Tacrolimus 1.5 mg PO BID initially, mycophenolate mofetil 1000 mg PO BID, prednisone taper starting 20 mg daily	Trough target 10-12 early post transplant; CBC/CMP twice weekly initially.
Prophylaxis	Valganciclovir 450 mg daily, TMP-SMX SS Mon/Wed/Fri, nystatin QID	CMV D+/R+, PJP prophylaxis, fungal prophylaxis.
Cardiac allograft vasculopathy prevention	Aspirin 81 mg daily, pravastatin 20 mg nightly	Start when platelets stable and bleeding risk acceptable.
BP / renal	Amlodipine 5 mg daily, furosemide 20 mg PRN weight gain per transplant instructions	ACE/ARB deferred due AKI and tacrolimus.
Diabetes	Insulin glargine 12 units nightly, insulin aspart correction with meals	Prednisone taper expected to reduce insulin needs.
GI / bone / electrolytes	Pantoprazole, calcium/vitamin D, magnesium oxide	Steroid and tacrolimus adverse-effect prevention.

Post-discharge orders

Order type	Details
Laboratory	CBC with differential, CMP, magnesium, phosphorus, tacrolimus trough every Monday/Thursday until transplant clinic changes interval.
Surveillance	Endomyocardial biopsy and right-heart cath per transplant calendar; CMV PCR every 2 weeks while on valganciclovir.
Home health	Skilled nursing for medication teaching, incision check, vitals log review, infection screening.
Rehabilitation	Home PT/OT initially, transition to outpatient PT/OT when safe transportation and endurance improve.
Equipment	Single-point cane, shower chair, BP cuff, glucometer, CPAP continuation. All latex-free supplies where applicable.

Discharge functional status

At discharge, patient ambulated 250 feet with rolling walker and standby assist, climbed 4 steps with rail and contact guard assist, required setup for upper body dressing, minimal assist for lower body dressing, and supervision for shower transfers. Residual left-sided weakness from prior stroke and marked cardiopulmonary deconditioning remained barriers to independence.

Operative Report - Orthotopic Heart Transplant - Page 1

Patient	John Alvarez	MRN	14332
Date of surgery	02/07/2026	Surgeon	Harrison Menon, MD
Assistants	L. Chen, MD; S. Patel, PA-C	Anesthesia	General endotracheal anesthesia
Pre-op diagnosis	End-stage ischemic cardiomyopathy with cardiogenic shock on IABP support	Post-op diagnosis	Same; status post orthotopic heart transplant

Procedures performed

Procedure
Redo median sternotomy with extensive lysis of adhesions.
Explantation of native heart and removal of ICD generator and transvenous leads.
Orthotopic heart transplantation using bicaval technique.
Back-table donor-heart preparation.
Intraoperative transesophageal echocardiography.
Placement of mediastinal and pleural chest tubes and temporary epicardial pacing wires.

Operative indications

The patient is a 70-year-old male with severe ischemic cardiomyopathy, prior coronary bypass surgery, chronic systolic heart failure, recurrent decompensation, and cardiogenic shock requiring intra-aortic balloon pump support. He had completed transplant evaluation and accepted a suitable donor organ. Risks, benefits, and alternatives were reviewed with the patient and spouse. Latex-free operating room precautions were used because of documented latex allergy. Penicillin allergy was present; perioperative antibiotics were selected in consultation with transplant infectious disease.

Operative metrics

Metric	Value
Cardiopulmonary bypass time	193 minutes
Donor ischemic time	181 minutes
Aortic cross-clamp time	86 minutes for donor implantation phase
Estimated blood loss	Cell-saver assisted; estimated 900 mL surgical field loss
Blood products	2 units PRBC, 1 platelet pack, cell saver return 475 mL
Drains / wires	2 mediastinal chest tubes, 1 right pleural chest tube, atrial and ventricular pacing wires
Specimens	Native heart, ICD generator, ICD leads sent per protocol

Operative Report - Orthotopic Heart Transplant - Page 2

Description of procedure

After confirming identity, donor organ availability, consent, allergy precautions, and transplant checklist, the patient was brought to the operating room and placed supine. General anesthesia was induced. Invasive monitoring lines were placed. A redo median sternotomy was performed. Dense mediastinal adhesions from prior coronary bypass were carefully dissected. Heparin was administered and cardiopulmonary bypass established with aortic and bicaval cannulation. The recipient heart was explanted using standard bicaval technique. ICD generator and leads were removed. The donor heart was inspected and prepared on the back table. Left atrial, inferior vena cava, superior vena cava, pulmonary artery, and aortic anastomoses were performed. The heart was de-aired and reperfused. Temporary pacing wires were placed. The patient separated from bypass on low-dose epinephrine and dobutamine. TEE showed normal left ventricular function, mildly reduced right ventricular function that improved with inotrope support, and no significant valvular abnormality. Hemostasis was achieved and the sternum closed with wires. All counts were correct.

Findings

Finding area	Details		
Recipient heart	Severely dilated, globally hypokinetic native heart with prior surgical adhesions.		
Donor organ	Good quality donor heart with no gross structural abnormality on back-table inspection.		
Post-implant function	Immediate graft function acceptable; sinus rhythm with temporary pacing support available.		
Complications	No intraoperative complication documented. No anaphylactic or latex exposure event documented.		
Disposition	Transferred intubated to CVICU in stable but critical condition on epinephrine, dobutamine, inhaled pulmonary vasodilator, insulin infusion, and IABP support.		
Electronic signature	Electronically signed by Harrison Menon, MD	Date/Time	02/07/2026 23:18
Authentication statement	This synthetic note was generated for OmniScribe testing and is not a real clinical record.	Status	Signed

Anesthesia, Perfusion, and Immediate Postoperative Summary

Anesthesia start	02/07/2026 06:42	Anesthesia end	02/07/2026 22:04
Airway	ETT 8.0, video laryngoscopy, 1 attempt	Lines	Right IJ introducer/Swan, left radial arterial line, Foley, latex-free precautions
ASA class	IV-E	Antibiotics	Non-penicillin regimen per ID due severe penicillin allergy

Intraoperative events and management

Domain	Details
Hemodynamics	Pre-bypass low cardiac output supported by IABP and vasoactive medications. Post-bypass required epinephrine 0.04 mcg/kg/min, dobutamine 5 mcg/kg/min, norepinephrine low dose, and inhaled epoprostenol.
Ventilation	Lung-protective settings. No bronchospasm documented. Extubation deferred to ICU because of procedure duration and vasoactive support.
TEE	Post-implant: LV normal size and systolic function EF estimated 60%; RV mildly reduced but improving; no pericardial effusion; no significant valve lesion.
Glucose	Insulin infusion used for stress and steroid-associated hyperglycemia.
Renal	Urine output 640 mL during case. Creatinine pre-op 1.58. Nephrotoxin avoidance noted.
Blood management	Cell saver, PRBC, platelets; fibrinogen acceptable after replacement. No uncontrolled bleeding at closure.
Allergy precautions	Latex-free cart verified; no penicillin-class antibiotic administered; no anaphylaxis or rash observed intraoperatively.

Perfusion record highlights

Parameter	Value
Lowest temperature	32.4 C
Lowest hematocrit on bypass	22%
Lowest recorded pH	7.32
Cardioplegia / preservation	Donor organ preserved per transplant protocol; reperfusion uneventful.
Wean from bypass	Successful on first attempt with inotrope and temporary pacing support available.

Cardiovascular ICU Progress Note - POD0 - 02/07/2026 23:45

Service	CVICU / transplant surgery	Provider	Nora Bell, MD
Post-op day	0	Status	Signed 02/08/2026 00:31

Subjective / interval

Arrived from OR intubated and sedated after OHT. On epinephrine, dobutamine, low-dose norepinephrine, inhaled epoprostenol, insulin infusion, propofol/fentanyl sedation. IABP remains 1:1. Chest tubes with expected sanguineous output. No immediate concern for surgical bleeding. Spouse updated by transplant surgery.

Objective

Domain	Finding
Vitals / support	T 35.9 C warming; HR 108 paced/NSR; MAP 72 on pressors; ventilator AC/VC Fio2 40%, PEEP 5; SpO2 99%.
Hemodynamics	CVP 10; PA 34/18; CI 2.4 on inotrope; lactate 2.8 down-trending.
Exam	Intubated, sedated. Sternotomy dressing dry. Lungs equal. Abdomen soft. Extremities warm with dopplerable pulses.
Lines/tubes	ETT, right IJ Swan, left radial arterial line, Foley, mediastinal and pleural chest tubes, pacing wires, right femoral IABP.
Labs	WBC 12.4, Hgb 8.9, Plt 136, INR 1.4, Cr 1.62, K 4.8, Mg 2.3, glucose 196.

Assessment / plan

Problem	Plan
Post OHT graft support	Continue inotrope support, monitor CI and lactate, maintain MAP 65-80, pacing wires available.
Respiratory failure	Continue ventilator overnight; spontaneous awakening/breathing trial when stable.
Immunosuppression	Steroid induction per protocol; begin mycophenolate; tacrolimus when renal function allows.
Bleeding/anemia	Trend chest tube output and CBC/coags; transfuse per transplant thresholds.
Allergies	Maintain latex-free precautions; penicillin allergy noted on ICU medication profile.

Cardiovascular ICU Progress Note - POD1 - 02/08/2026

Post-op day	1	Provider	Nora Bell, MD
Major events	Extubated, vasopressors decreasing, mobilization initiated	Status	Signed 02/08/2026 18:22

Interval events

Passed spontaneous breathing trial and extubated at 11:15 to nasal cannula 4 L. Awake, follows commands, recognizes spouse. Mild left-sided baseline weakness noted. Pain controlled with acetaminophen and low-dose opioid. IABP continued with plan for removal if CI stable. Chest tube output slowing. No fever.

Vitals, labs, and functional status

Item	Result
Vitals	T 37.4 C, HR 104, BP 118/64 on low-dose support, RR 18, SpO2 96% on 4 L NC.
Hemodynamics	CVP 8, PA 30/16, CI 2.7, lactate 1.9.
Labs	WBC 11.8, Hgb 8.7, Plt 122, Na 138, K 4.9, BUN 32, Cr 1.74, Mg 2.0, glucose 165-240 on insulin infusion.
Therapy	Sat edge of bed with PT/OT and two-person assist; stood for 20 seconds with walker; limited by fatigue.
Swallow	Bedside swallow screen passed for ice chips and sips; advanced to cardiac/diabetic diet as tolerated.

Plan by system

System	Plan
Cardiac	Wean norepinephrine; continue dobutamine and epinephrine; evaluate for IABP removal tomorrow.
Pulmonary	Incentive spirometry every hour while awake; pulmonary hygiene; wean oxygen.
Renal	AKI monitoring; maintain euvolemia; avoid nephrotoxins; renally dose valganciclovir.
Endocrine	Transition insulin infusion to basal/bolus when stable oral intake.
Neurologic	Delirium prevention; reorient frequently; monitor for new stroke signs.
Rehab	PT/OT daily; sternal precautions; latex-free gait belt and therapy materials.

Cardiovascular ICU Progress Note - POD4 - 02/11/2026

Post-op day	4	Provider	J. Morales, NP
Major events	IABP removed POD2; delirium improving; ambulation started	Status	Signed 02/11/2026 16:05

Interval history

No acute overnight events. IABP removed 02/09 without vascular complication. Patient oriented to self and hospital, intermittently confused at night but redirectable. Oxygen weaned to room air during daytime. Dobutamine taper ongoing. Chest tube output minimal. Ambulated 75 feet with front-wheeled walker and moderate assist of two due fatigue and left-sided weakness.

Problem	Assessment	Plan
Graft function	Clinically stable, CI 2.8, lactate normalized. Mild sinus tachycardia expected post transplant.	Continue dobutamine taper. Daily EKG. First biopsy per protocol.
Respiratory	Atelectasis on CXR, no pneumonia.	Incentive spirometry, walking, CPAP at night if tolerated.
Renal	Cr 1.91 peak, urine output adequate.	Hold diuretics today, maintain MAP, avoid NSAIDs.
Delirium	Hypoactive ICU delirium improving.	Sleep protocol, minimize narcotics, family presence.
Immunosuppression	Tacrolimus initiated; mycophenolate and prednisone continue.	Daily trough after steady state; monitor tremor/renal function.
Infection prophylaxis	No fever. WBC 9.8.	Continue valganciclovir, TMP-SMX, nystatin.
Rehabilitation	Severe deconditioning plus prior CVA deficits.	PT/OT daily; document endurance, transfers, and stair readiness.

Selected labs 02/11/2026

WBC	9.8 K/uL	Hgb	8.4 g/dL	Platelets	148 K/uL
Na	137 mmol/L	K	4.7 mmol/L	Creatinine	1.91 mg/dL
Tacrolimus trough	4.1 ng/mL	Glucose range	142-268 mg/dL	Lactate	1.2 mmol/L

Transplant Step-down Progress and Discharge Education - 02/23/2026

Unit	Transplant step-down	Provider	Mei Santos, PA-C
Hospital day	20	Post-op day	16

Progress summary

Patient is tolerating oral intake and walking short distances on the unit with front-wheeled walker and standby to contact guard assist. Sternotomy incision clean and dry. Temporary pacing wires removed without complication. Chest tubes removed. Renal function improving. Patient and spouse completed first medication-teaching session and demonstrated understanding of empty stomach tacrolimus timing, trough lab timing, missed-dose instructions, and infection precautions.

Teaching checklist

Topic	Patient / spouse understanding	Follow-up needed
Immunosuppression schedule	Spouse correctly names tacrolimus, mycophenolate, prednisone and states no missed doses.	Repeat teach-back before discharge.
Tacrolimus trough labs	Understands morning dose held until blood draw on lab days.	Provide written calendar.
Infection precautions	Understands temperature check and call threshold. No live vaccines.	Reinforce food safety.
Diabetes care	Patient can check glucose; spouse can administer insulin if needed.	Diabetes educator to review sliding scale.
Rehab safety	Uses walker with cues for posture. Prior left weakness persists.	Stair training before discharge.
Allergies	Penicillin, bee sting, latex reviewed.	Latex-free home health supplies requested.

Functional update

Task	Current level	Barriers / cues
Bed mobility	Supervision	Sternal precautions; log roll reminders.
Sit to stand	Standby assist	Uses arms less; mild posterior lean.
Ambulation	180 feet with front-wheeled walker, standby assist	Fatigue after 120 feet; HR 112, SpO2 96%.
Stairs	4 steps with rail, contact guard assist	Left leg weakness and endurance.
ADLs	Setup upper body dressing; minimal assist lower body dressing	Left hand fine motor slowness.

Consult Notes - Transplant Infectious Disease and Nephrology

Transplant Infectious Disease Consult - 02/05/2026 and update 02/12/2026

Question	Recommendations
Perioperative antibiotics with severe penicillin allergy	Use non-penicillin perioperative prophylaxis per transplant protocol. Avoid penicillin-class agents unless allergy evaluation/desensitization performed. Documented latex precautions.
CMV / opportunistic infection prophylaxis	CMV donor positive/recipient positive: valganciclovir renal dosing. PJP: TMP-SMX SS three times weekly. Fungal: nystatin while on high-dose steroids.
Pre-transplant screening	Blood cultures negative. Urine culture mixed flora/contaminant. No active dental or skin infection.
Fever plan	For fever or sepsis, obtain blood cultures, chest imaging, urinalysis/culture, viral PCR as clinically indicated; consider broad non-penicillin regimen with transplant ID input.

Nephrology Consult - AKI after transplant - 02/10/2026

Assessment item	Details
Clinical assessment	Non-oliguric AKI on CKD 3a, likely multifactorial from cardiogenic shock, bypass time, vasopressors, and tacrolimus initiation. No indication for dialysis.
Creatinine trend	Baseline 1.25-1.35 prior to admission; post-op 1.62 -> 1.74 -> 1.91 peak -> 1.70 by 02/14.
Urinalysis	Trace protein, no casts reported, no infection.
Plan	Maintain perfusion pressure, avoid NSAIDs/IV contrast if possible, renally dose valganciclovir, monitor tacrolimus troughs, daily BMP/Mg/Phos, strict intake/output.
Discharge recommendation	Outpatient CMP twice weekly initially. Educate patient on hydration and sick-day medication questions.

Acute Hospital Discharge Summary - Ischemic CVA - Page 1

Hospital	Columbia River Acute Hospital	Admission	04/18/2025
Discharge	04/23/2025	Attending	Priya Raman, MD - Neurology
Principal diagnosis	Acute ischemic stroke, right middle cerebral artery territory	Disposition	Inpatient rehabilitation facility

Presentation

Patient presented with left facial droop, left arm weakness, mild dysarthria, left-sided inattention, and gait instability. Last known well was approximately 10 hours before arrival, outside thrombolysis window. Initial NIH Stroke Scale score 9. CT head showed no hemorrhage. CTA showed right M2 occlusion with distal reconstitution. CT perfusion suggested completed infarct with limited salvageable tissue; neurointerventional team did not perform thrombectomy.

Hospital course

Hospital issue	Course and discharge status
Right MCA ischemic stroke	MRI confirmed acute infarct in right MCA territory without hemorrhagic conversion. Deficits improved but residual left weakness and fine motor impairment remained.
Etiology	Presumed cardioembolic in setting of atrial fibrillation and severe ischemic cardiomyopathy. Carotid disease mild and not felt causative.
Antithrombotic therapy	Aspirin used acutely. Anticoagulation initiated after repeat head CT stable. Discharged on apixaban at that time; later medication regimen changed after transplant.
Dysphagia	Initial mild oropharyngeal dysphagia improved. Discharge diet regular texture with thin liquids and upright positioning.
Rehabilitation need	PT/OT recommended intensive inpatient rehab due left hemiparesis, balance deficits, and ADL assistance needs.
Blood pressure	Permissive hypertension initially, then gradual control. Long-term BP goal less than 130/80.

Acute Hospital Discharge Summary - Ischemic CVA - Page 2

Neurologic exam at acute discharge 04/23/2025

Domain	Finding
Mental status	Alert and oriented. Mild slowed processing. Follows 2-step commands.
Cranial nerves	Mild left lower facial droop; speech intelligible with mild dysarthria.
Motor	Right side 5/5. Left shoulder abduction 4-/5, left grip 3+/5, left hip flexion 4-/5, left ankle dorsiflexion 4/5.
Sensation / neglect	Light touch intact; left inattention improved but still misses left-sided visual scanning targets when fatigued.
Coordination	Left finger-to-nose slowed; no severe ataxia.
Gait	Ambulates 40 feet with rolling walker and moderate assist; unsafe for independent mobility.

Discharge medications from acute stroke admission

Medication at 04/23/2025 discharge	Purpose / notes
Apixaban 5 mg PO BID	Stroke prevention for atrial fibrillation / cardioembolic risk at that time.
Atorvastatin 80 mg PO nightly	High-intensity statin after ischemic stroke.
Carvedilol, sacubitril/valsartan, spironolactone, furosemide	Heart failure regimen before later transplant course; no longer current as listed.
Sertraline 25 mg daily started then titrated	Post-stroke mood symptoms.
Bowel regimen and acetaminophen PRN	Supportive medications.

Follow-up and secondary prevention at 2025 discharge

Follow-up	Instructions
Neurology	Follow-up in 4-6 weeks with repeat assessment and anticoagulation tolerance review.
Cardiology	HF clinic follow-up due EF near 20% and arrhythmia history.
Therapy	Acute inpatient rehab then outpatient therapy for left hemiparesis, balance, ADL retraining.
Risk-factor control	BP control, diabetes screening, LDL goal less than 70, smoking avoidance, medication adherence.

Electronic signature	Electronically signed by Priya Raman, MD	Date/Time	04/23/2025 13:06
Authentication statement	This synthetic note was generated for OmniScribe testing and is not a real clinical record.	Status	Signed

Neurology Acute Care Progress Note - CVA Admission - 04/20/2025

Service	Neurology	Provider	Priya Raman, MD
Hospital day	3	NIHSS trend	Initial 9; current 5

Subjective

Patient reports frustration with left arm weakness but notes speech is clearer. No headache, new vision loss, chest pain, or dyspnea. Spouse at bedside. Patient asks when he can walk alone; safety education provided.

Neurologic exam

NIHSS element	Finding / score
Level of consciousness	Alert, answers questions correctly - 0
Best gaze / visual fields	No forced gaze; partial left visual inattention - 1
Facial palsy	Minor left facial droop - 1
Motor left arm	Drift, does not hit bed - 1
Motor left leg	Drift, does not hit bed - 1
Limb ataxia	Present left upper extremity - 1
Language / dysarthria	No aphasia; mild dysarthria - 1
Extinction / inattention	Mild left neglect - 1

Assessment and plan

Problem	Plan
Right MCA ischemic stroke	Continue neuro checks every 4 hours, telemetry, repeat CT head before anticoagulation start.
Stroke mechanism	Likely cardioembolic. Echo shows severe LV systolic dysfunction; telemetry with atrial fibrillation.
Antithrombotic plan	Aspirin now. Start apixaban after stable repeat imaging and swallowing clearance.
Rehabilitation	PT/OT/SLP daily; inpatient rehab referral started.
Swallow / nutrition	Mechanical soft initially; SLP to reassess. Aspiration precautions.
Risk factor	High-intensity statin, BP plan after acute period, diabetes screening, smoking status confirmed negative.

CVA Admission Therapy and Swallow Consult Summary - 04/19/2025 to 04/23/2025

Physical therapy acute evaluation 04/19/2025

Domain	Finding
Bed mobility	Moderate assist supine to sit due left arm weakness and poor trunk control.
Transfers	Moderate assist sit-to-stand with front-wheeled walker. Left knee buckling when fatigued.
Gait	12 feet with rolling walker, moderate assist, decreased left foot clearance, left neglect causing obstacle contact.
Balance	Sitting balance fair, standing balance poor. High fall risk.
PT recommendation	Inpatient rehabilitation, 3 hours/day if medically tolerated.

Occupational therapy acute evaluation 04/20/2025

ADL / function	Status
Feeding / grooming	Setup to minimal assist; misses objects on left side of tray.
Upper body dressing	Moderate assist; left arm placement and sequencing cues.
Lower body dressing	Max assist for socks/pants due balance and left hand weakness.
Toileting	Moderate assist with transfers and clothing management.
Cognition / scanning	Mild left neglect; benefits from visual anchor and verbal cueing.
OT recommendation	Inpatient rehabilitation with focus on ADLs, left upper extremity neuro re-education, scanning, safety.

Speech-language pathology swallow update 04/22/2025

Item	Finding / recommendation
Swallow status	Improved oral control; no overt aspiration with thin liquids during supervised trials.
Diet	Regular texture, thin liquids, medications whole with water as tolerated.
Precautions	Upright at 90 degrees, slow rate, small bites/sips, check left pocketing when fatigued.
Cognitive communication	Mild attention/executive slowing; outpatient or inpatient rehab cognitive strategies recommended.

Recent Laboratory Results - CBC, Chemistry, Renal Function - 05/21/2026

Test	Result	Flag	Reference range	Units	Date	Comment
WBC	6.2		4.0-11.0	K/uL	05/21/2026	Stable on mycophenolate.
RBC	3.84	L	4.40-5.90	M/uL	05/21/2026	Post-op/CKD anemia.
Hemoglobin	11.8	L	13.5-17.5	g/dL	05/21/2026	Improved from discharge.
Hematocrit	35.9	L	41-53	%	05/21/2026	Stable.
MCV	93.5		80-100	fL	05/21/2026	Normocytic.
Platelets	226		150-400	K/uL	05/21/2026	Normal.
Neutrophils absolute	4.8		1.8-7.7	K/uL	05/21/2026	No neutropenia.
Sodium	139		135-145	mmol/L	05/21/2026	Normal.
Potassium	4.6		3.5-5.1	mmol/L	05/21/2026	High-normal; monitor with CKD/tacrolimus.
Chloride	105		98-107	mmol/L	05/21/2026	Normal.
CO2 / bicarbonate	22		22-29	mmol/L	05/21/2026	Low-normal.
BUN	31	H	8-23	mg/dL	05/21/2026	Likely CKD/dehydration component.
Creatinine	1.42	H	0.70-1.30	mg/dL	05/21/2026	CKD 3a; stable vs transplant clinic.
eGFR	53	L	>60	mL/min/1.73m ²	05/21/2026	Dose meds by renal function.
Glucose	142	H	70-99 fasting	mg/dL	05/21/2026	Steroid-related hyperglycemia.
Calcium	9.1		8.6-10.2	mg/dL	05/21/2026	Normal.
Magnesium	1.6	L	1.7-2.4	mg/dL	05/21/2026	On magnesium oxide.
Phosphorus	3.4		2.5-4.5	mg/dL	05/21/2026	Normal.

Lab interpretation summary

Renal function is stable but abnormal for age after transplant and tacrolimus exposure. Magnesium is mildly low. CBC shows improving normocytic anemia without leukopenia or thrombocytopenia. Glucose remains above goal during prednisone taper.

Recent Laboratory Results - Transplant, Endocrine, Lipids, Infection Surveillance

Test	Result	Flag	Reference / target	Units	Date	Comment
Tacrolimus trough	10.2		8-12	ng/mL	05/21/2026	At target. Drawn before AM dose.
Tacrolimus trough	9.7		8-12	ng/mL	05/14/2026	At target.
Tacrolimus trough	14.4	H	8-12	ng/mL	05/07/2026	Dose reduced after high level and tremor.
Tacrolimus trough	7.8	L	8-12	ng/mL	04/28/2026	Dose adjusted upward at transplant clinic.
AST	18		0-40	U/L	05/21/2026	Normal.
ALT	27		0-41	U/L	05/21/2026	Normal.
Alkaline phosphatase	84		40-129	U/L	05/21/2026	Normal.
Albumin	3.9		3.5-5.2	g/dL	05/21/2026	Normal.
Hemoglobin A1c	7.6	H	<7.0 individualized	%	05/21/2026	Steroid effect.
LDL cholesterol	72		<70 preferred	mg/dL	05/21/2026	Near goal.
HDL cholesterol	41		>40	mg/dL	05/21/2026	Acceptable.
Triglycerides	189	H	<150	mg/dL	05/21/2026	Mildly elevated.
TSH	2.4		0.4-4.5	mIU/L	05/21/2026	Normal.
BNP	142	H	<100	pg/mL	05/21/2026	Mild elevation, echo stable.
CMV PCR	Not detected		Not detected	IU/mL	05/14/2026	Continue prophylaxis.
EBV PCR	Not detected		Not detected	copies/mL	05/14/2026	Surveillance.
BK virus urine PCR	Not detected		Not detected	copies/mL	05/14/2026	Surveillance.
Urinalysis protein	Trace	A	Negative		05/21/2026	No infection markers.

Transplant lab note

Tacrolimus levels reflect a dose-adjustment period in late April and early May. Most recent trough is in target range. Viral surveillance is negative in this synthetic sample.

Imaging and Diagnostic Reports - Brain / Stroke History

CT Head without contrast - 04/18/2025 09:24

Indication	Acute left facial droop and left arm weakness.
Technique	Noncontrast CT head.
Findings	No acute intracranial hemorrhage. Early loss of gray-white differentiation in right frontal opercular region. Chronic microvascular ischemic changes. No hydrocephalus or mass effect.
Impression	No hemorrhage. Early right MCA territory ischemic change suspected. Correlate with CTA/CT perfusion.

CTA Head/Neck with perfusion - 04/18/2025 09:41

Findings	Right M2 branch occlusion with distal reconstitution. No cervical carotid occlusion. Approximately 30% left internal carotid bulb stenosis and 25% right internal carotid bulb stenosis. CT perfusion demonstrates core infarct in right MCA distribution with small mismatch volume.
Impression	Right M2 occlusion with limited penumbra; mild bilateral carotid atherosclerosis. Neurointerventional team notified at time of exam.

MRI Brain without contrast - 04/19/2025 14:32

Findings	Restricted diffusion in right frontal operculum, right insula, and scattered right MCA cortical/subcortical territories. No hemorrhagic conversion. Mild chronic small vessel disease.
Impression	Acute/subacute right MCA territory infarcts without hemorrhagic transformation.

CT Head without contrast - 04/02/2026

Findings / impression	Chronic right MCA territory encephalomalacia consistent with prior infarct. No acute hemorrhage, mass effect, or new territorial infarct.
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Imaging and Diagnostic Reports - Cardiac and Chest

Transthoracic echocardiogram - 05/10/2026

Parameter	Result
LV size/function	Normal LV size, EF 63% by biplane method. No regional wall motion abnormality.
RV	Normal size and systolic function.
Valves	Trace tricuspid regurgitation; no significant stenosis or regurgitation.
Pericardium	No pericardial effusion.
Impression	Normal biventricular function in transplanted heart; no significant valve disease.

Chest X-ray PA/lateral - 05/21/2026

Findings	Post median sternotomy and transplant changes. Cardiomeastinal silhouette not enlarged. Minimal left basilar linear atelectatic scar. No focal pneumonia, pleural effusion, pneumothorax, or pulmonary edema.
Impression	No acute cardiopulmonary abnormality. Stable post-transplant changes.

CT Chest without contrast - 03/18/2026

Indication	Post-transplant cough and low-grade temperature at home; evaluate infection.
Findings	Mild dependent atelectasis. No consolidation, cavitary lesion, pleural effusion, or mediastinal collection. Sternotomy wires intact. No suspicious lymphadenopathy.
Impression	No CT evidence of pneumonia or postoperative mediastinal abscess. Mild atelectasis.

EKG - 05/21/2026

Result	Sinus rhythm, rate 93, right bundle branch block pattern seen on prior post-transplant tracing, QTc 462 ms, no acute ischemic change.
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Pathology and Transplant Surveillance Reports

Endomyocardial biopsy - 02/21/2026

Specimen	Right ventricular endomyocardial biopsy, 4 fragments.
Microscopic diagnosis	No significant acute cellular rejection, ISHLT grade 0R. No histologic evidence of antibody-mediated rejection.
Immunostains	C4d negative, CD68 without pathologic intravascular macrophage accumulation.
Final diagnosis	0R, pAMR0.

Endomyocardial biopsy - 02/28/2026

Specimen	Right ventricular biopsy, 5 fragments.
Microscopic diagnosis	Mild focal interstitial lymphocytic infiltrate without myocyte damage.
Final diagnosis	Mild acute cellular rejection, ISHLT 1R; pAMR0. Clinical team elected observation and tacrolimus optimization.

Endomyocardial biopsy - 05/10/2026

Specimen	Right ventricular biopsy, 4 fragments.
Microscopic diagnosis	No acute cellular rejection. No features of antibody-mediated rejection.
Immunostains	C4d negative.
Final diagnosis	0R, pAMR0.

Donor-specific antibody screen - 05/10/2026

Result	No donor-specific HLA antibodies detected in this synthetic surveillance sample.
Recommendation	Continue routine surveillance per transplant protocol.

Medication Safety, Allergy Management, and Patient Education Note - 05/21/2026

Medication reconciliation method

Medication list reconciled with patient, spouse, transplant discharge list, transplant clinic medication card, pharmacy fill history, and pillbox review. Spouse uses a color-coded checklist and confirms tacrolimus is taken after trough labs on lab mornings. Patient denies missed doses in the prior 14 days. Primary concern is confusion around hydralazine PRN thresholds and insulin correction scale; written instructions reprinted.

Drug interaction and safety review

Risk area	Action / education
Tacrolimus interactions	Avoid grapefruit, pomelo, St. John wort, and new antibiotics/antifungals without transplant approval. Diltiazem/verapamil/macrolides/azoles can increase tacrolimus levels.
Nephrotoxins	Avoid NSAIDs such as ibuprofen/naproxen unless transplant/nephrology directs. Hydrate during illness and call clinic for vomiting/diarrhea.
Steroid hyperglycemia	Reviewed prednisone timing, glucose checks before meals and bedtime, hypoglycemia rule of 15.
Aspirin / bleeding	Report black stools, persistent nosebleeds, or large unexplained bruising.
Penicillin allergy	Do not self-administer leftover antibiotics. Allergy visible on chart and discharge lists.
Bee sting allergy	Epinephrine auto-injector renewed. Teach-back: inject lateral thigh and call emergency services.
Latex allergy	Clinic room flagged latex-free. Rehab team notified to avoid latex bands and gloves.

Patient teach-back

Patient and spouse correctly explain: fever call threshold, tacrolimus trough timing, need to call before new medications, signs of rejection/infection, avoidance of latex supplies, and use of epinephrine for bee sting anaphylaxis. Further education needed for insulin correction scale; diabetes educator appointment requested.

Outpatient Physical Therapy Initial Evaluation - 04/15/2026

Facility	Lake Roosevelt Rehabilitation Services	Therapist	Dana Kim, PT, DPT
Referral diagnosis	Deconditioning after heart transplant; gait instability; residual left hemiparesis after CVA	Plan	2 visits/week for 8 weeks
Precautions	Heart transplant precautions, immunosuppression, latex allergy, fall risk, monitor BP/HR/RPE, prior sternotomy	Certification period	04/15/2026 - 06/14/2026

Subjective

Patient reports fatigue, reduced walking tolerance, and fear of falling after February heart transplant and prior 2025 stroke. He walks household distances without device at times but uses single-point cane outdoors. Reports left leg feels heavy when tired. Goal: walk around clinic campus, resume light household chores, and reduce spouse assistance.

Objective measures

Measure	Score / finding	Interpretation
Timed Up and Go	18.6 seconds with single-point cane	Increased fall risk.
5 Times Sit-to-Stand	22.1 seconds from standard chair, uses hands lightly	Reduced LE power/endurance.
6 Minute Walk Test	720 feet with cane; 2 standing rests; RPE 13; HR 94 -> 116; SpO2 98 -> 96%	Reduced aerobic capacity.
Berg Balance Scale	41/56	Moderate fall risk.
Gait	Reduced left stance time, decreased left foot clearance, mild trunk flexion, slower turns.	
Strength	Right LE mostly 4+/5. Left hip flexion 4-/5, knee extension 4/5, ankle dorsiflexion 4/5.	
Pain	0-2/10 sternotomy tightness with reaching; no chest pain.	

Assessment and goals

Assessment	Patient presents with post-transplant deconditioning, impaired dynamic balance, residual left hemiparesis, reduced endurance, and high fall risk. Good rehab potential with structured progressive program.		
Short-term goals	TUG less than 15 sec; 6MWT at least 850 ft; independent HEP; no falls in 4 weeks; demonstrate safe BP/HR/RPE monitoring.		
Long-term goals	TUG less than 12 sec; 6MWT at least 1050 ft; Berg at least 49/56; walk community distances with least restrictive device; resume light household tasks within transplant restrictions.		
Electronic signature	Electronically signed by Dana Kim, PT, DPT	Date/Time	04/15/2026 15:08
Authentication statement	This synthetic note was generated for OmniScribe testing and is not a real clinical record.	Status	Signed

Outpatient Physical Therapy Daily Note - 05/08/2026

Visit	PT visit 5 of 16	Therapist	Dana Kim, PT, DPT
Subjective	Reports walking 7 minutes twice daily. No falls. Mild tremor after tacrolimus dose. Denies chest pain or fever.	Pain	0/10

Pre/post vitals

Time	BP	HR	SpO2	RPE	Notes
Pre-session	138/82	96	98%	9	No symptoms.
After gait intervals	148/84	112	97%	13	Appropriate response; no chest pain.
End of session	132/78	98	98%	10	Recovered in 4 minutes.

Interventions

CPT / intervention	Details	Response
97110 Therapeutic exercise - 18 min	Seated warm-up, sit-to-stand 3x8 from 20-inch surface, standing hip abduction 2x10, heel raises 2x12.	Fatigues but maintains form with rest breaks.
97116 Gait training - 16 min	Cane sequencing, 4x120 ft intervals, turn training, left foot clearance cues.	Improved left step height with verbal cue. No loss of balance.
97112 Neuromuscular re-ed - 12 min	Static stance narrow base, step taps to 4-inch step, visual scanning left while standing.	Requires close supervision; mild instability with left scanning.
97530 Therapeutic activity - 8 min	Functional reaching within sternotomy comfort zone and carrying light object 2 lb.	No sternotomy pain.

Assessment / plan

Patient demonstrates improving activity tolerance. Continue graded endurance, balance, left LE strengthening, gait safety, and education on transplant precautions. Latex-free equipment used; no latex resistance bands.

Electronic signature	Electronically signed by Dana Kim, PT, DPT	Date/Time	05/08/2026 14:36
Authentication statement	This synthetic note was generated for OmniScribe testing and is not a real clinical record.	Status	Signed

Outpatient Physical Therapy Progress Note - 05/24/2026

Reporting period	04/15/2026 - 05/24/2026	Visits completed	9 of 16
Therapist	Dana Kim, PT, DPT	Referring provider	Alina Martin, MD

Progress measures

Measure	Initial 04/15	Current 05/24	Progress interpretation
Timed Up and Go	18.6 sec with cane	14.8 sec with cane	Improved but still above low-fall-risk threshold.
5 Times Sit-to-Stand	22.1 sec	17.4 sec	Improved LE functional strength.
6 Minute Walk Test	720 ft	910 ft	Improved endurance; still below goal 1050 ft.
Berg Balance	41/56	46/56	Reduced fall risk but dynamic balance deficits remain.
Falls	0 reported	0 reported	Goal met to date.
Device use	Cane outdoors	Cane outdoors; no device indoors when rested	Progressing.

Clinical assessment

Patient is making measurable progress in gait speed, endurance, balance, and functional transfers. Residual limitations include left foot clearance when fatigued, reduced single-leg stability, reduced cardiovascular endurance after OHT, and need for external pacing to avoid overexertion. He remains medically complex due to immunosuppression, hypertension, diabetes, CKD, and transplant precautions. Continued skilled PT is medically necessary to progress community mobility and reduce fall risk.

Updated plan

Frequency	Continue 2x/week for 4 more weeks, then reassess.		
Interventions	Progressive aerobic intervals, LE strengthening, dynamic balance, gait training on varied surfaces, stair training, fall recovery education when cleared.		
Precautions	Monitor BP/HR/RPE, avoid maximal exertion, infection screening at start of session, latex-free equipment.		
Electronic signature	Electronically signed by Dana Kim, PT, DPT	Date/Time	05/24/2026 11:22
Authentication statement	This synthetic note was generated for OmniScribe testing and is not a real clinical record.	Status	Signed

Outpatient Occupational Therapy Initial Evaluation - 04/18/2026

Facility	Lake Roosevelt Rehabilitation Services	Therapist	Mika Torres, OTR/L
Referral diagnosis	Left upper-extremity weakness and ADL deficits after CVA; deconditioning after heart transplant	Plan	2 visits/week for 8 weeks
Precautions	Latex allergy, immunosuppression, fall risk, monitor fatigue, post-sternotomy recovery	Certification period	04/18/2026 - 06/17/2026

Occupational profile

Patient lives with spouse in single-level home with 4 steps to enter. Before stroke and transplant, patient managed yard tasks, drove short distances, and performed medication management independently. Currently spouse assists with pillbox, lower body dressing when fatigued, meal preparation, transportation, and monitoring vitals. Patient wants to improve left hand use, return to safe showering without hands-on help, and manage medications with fewer errors.

Objective measures

Measure / task	Right	Left	Interpretation
Grip strength	62 lb	32 lb	Left weakness limits containers and cane confidence.
Lateral pinch	16 lb	8 lb	Reduced left pinch.
9-Hole Peg Test	27 sec	48 sec	Left fine motor delay.
Shoulder flexion AROM	145 deg	126 deg	Mild limitation; avoid aggressive chest strain.
QuickDASH	N/A	47.7%	Moderate perceived upper-limb disability.
MoCA screening	25/30	N/A	Mild attention/executive weakness; recommend strategies.

ADL/IADL status

Activity	Current level
Bathing	Supervision with shower chair; spouse nearby due fatigue/fall risk.
Dressing	Independent upper body with extra time; minimal assist compression socks/shoes on bad fatigue days.
Meal prep	Light snack setup; not cooking at stove independently.
Medication management	Spouse manages pillbox; patient participates but misses dose-time details without checklist.
Driving	Not driving; pending medical and functional clearance.

Electronic signature	Electronically signed by Mika Torres, OTR/L	Date/Time	04/18/2026 10:14
Authentication statement	This synthetic note was generated for OmniScribe testing and is not a real clinical record.	Status	Signed

Outpatient Occupational Therapy Daily and Progress Note - 05/22/2026

Visit	OT visit 8 of 16	Therapist	Mika Torres, OTR/L
Subjective	Patient reports he buttoned shirt using both hands for first time since transplant. Spouse still checks pillbox. No fever or new neurologic symptoms.	Pain	0/10

Interventions

CPT / intervention	Details	Response
97110 Therapeutic exercise - 12 min	Left shoulder/scapular AROM, light hand gripper, theraputty using latex-free materials.	No chest discomfort; mild fatigue.
97112 Neuromuscular re-ed - 18 min	Bilateral coordination, left visual scanning, object manipulation with varied textures, reach-place tasks below shoulder height.	Improved left attention and grip grading.
97535 Self-care management - 20 min	Medication checklist training, pillbox error detection, shower safety sequencing, energy conservation for dressing.	Patient identifies 4/5 simulated pillbox errors; needs cue for timing of tacrolimus on lab days.
97530 Therapeutic activity - 10 min	Kitchen counter task: carry light mug, open containers, wipe counter with left hand assist.	Completed with supervision; RPE 11.

Progress measures

Measure	Initial	Current	Interpretation
Left grip	32 lb	39 lb	Improved but still weaker than right.
Left 9-Hole Peg Test	48 sec	39 sec	Improved fine motor speed.
Medication task	Spouse full management	Patient fills 1-week mock box with 1 cue	Progressing toward shared management.
Shower transfer	Supervision	Distant supervision in clinic simulation	Improved safety.

Assessment / plan

Patient benefits from skilled OT for left UE coordination, medication-management safety, ADL independence, visual scanning, and fatigue management. Continue plan of care with latex-free materials and infection screening.

Electronic signature	Electronically signed by Mika Torres, OTR/L	Date/Time	05/22/2026 13:58
Authentication statement	This synthetic note was generated for OmniScribe testing and is not a real clinical record.	Status	Signed

Rehabilitation Plan of Care and Functional Summary

Interdisciplinary outpatient rehab problem list

Problem	Functional impact	Skilled rehab approach
Post-transplant deconditioning	Limited walking tolerance, fatigue with ADLs, needs pacing education.	Progressive aerobic intervals using RPE/HR/BP monitoring; energy conservation; home walking program.
Residual left hemiparesis	Reduced left foot clearance, weak left hand, slower dressing and medication tasks.	Strengthening, neuro re-education, task-specific ADL/IADL retraining, bilateral coordination.
Balance impairment	Fall risk in community, difficulty turning and stepping over obstacles.	Dynamic balance, gait training, obstacle negotiation, visual scanning with mobility.
Mild cognitive/executive deficits	Medication schedule errors possible, difficulty multitasking when fatigued.	External memory aids, checklist training, caregiver education, gradual complexity.
Immunosuppression and allergies	Infection risk and latex exposure risk in therapy setting.	Screening at every visit, clean equipment, latex-free supplies, avoid sick contacts.

Home exercise and self-management program

Program item	Instruction
Walking	Walk 8-12 minutes, 2-3 times/day, RPE 11-13, stop for dizziness, chest pain, unusual shortness of breath, or fever.
Strength	Sit-to-stand, heel raises, hip abduction, step taps; stop if form deteriorates or BP symptoms occur.
Balance	Counter-supported narrow stance and weight shifts; spouse nearby; no unsupervised challenging balance.
Upper extremity	Left hand coordination, coins/buttons, putty, reach-place tasks below shoulder until fully cleared.
Monitoring	Check BP/HR/glucose as instructed. Bring log to PCP/transplant visits.

Rehab discharge criteria

Anticipated outpatient rehab discharge when patient demonstrates safe community ambulation with least restrictive device, no recent falls, TUG below 12 seconds or clinically acceptable alternative, 6MWT over 1050 feet or individualized endurance goal, independent home program, improved medication-management safety, and stable vital-sign response within transplant-team limits.

Care Coordination, Home Health, DME, and Social Determinants

Home environment and support

Item	Details
Living situation	Single-level home with spouse, 4 entry steps with one rail. Bedroom and bathroom on main level.
Caregiver support	Spouse assists with transportation, medication setup, vital-sign logs, and meals. Adult daughter available intermittently.
Transportation	Spouse drives to transplant clinic and rehab. Patient not driving pending clearance.
Food access	Reports adequate food; instructed on transplant food safety and diabetes-consistent meals.
Health literacy	Patient understands general goals but needs written checklists due cognitive fatigue and complex medication schedule.
Spiritual/cultural preference	Patient requests family involvement in major decisions; no restriction on care documented.

DME and supplies

Equipment / supply	Status and instructions
Single-point cane	Used for outdoor/community ambulation. Check rubber tip wear.
Front-wheeled walker	Available at home for bad fatigue days; may be phased out as PT progresses.
Shower chair	Continue until balance/endurance improved and OT clears.
BP cuff	Use latex-free cuff. Log twice daily and with symptoms.
Glucometer	Check fasting and mealtime per diabetes plan.
CPAP	Continue nightly; clean equipment regularly due immunosuppression.
Epinephrine auto-injectors	Two devices prescribed; keep accessible at home and during outdoor activity.

Recent care-coordination actions

Date	Action
04/08/2026	PCP placed outpatient PT and OT referrals; allergy banner updated.
04/12/2026	Home health confirmed transition plan to outpatient rehab.
05/02/2026	Transplant nurse reviewed tacrolimus trough timing and lab calendar.
05/21/2026	PCP renewed epinephrine auto-injector and requested diabetes education appointment.

Immunizations, Preventive Care, and Advance Care Planning

Immunization and infectious precautions

Item	Status / note
Influenza vaccine	Documented 10/2025 before transplant. Annual inactivated vaccine recommended.
COVID-19 vaccine	Primary series and booster documented before transplant; transplant clinic to advise timing of additional boosters.
Pneumococcal vaccine	PCV20 documented 08/2024.
Shingles vaccine	Recombinant zoster series completed 2024. No live vaccines post transplant.
Tdap	Documented 07/2022.
Live vaccines	Contraindicated while immunosuppressed unless transplant team gives explicit direction.

Preventive care / chronic disease monitoring

Area	Plan
Diabetes	A1c every 3 months while steroid taper continues; annual foot/eye/kidney screening.
Hypertension	Home BP log; target less than 130/80 if tolerated.
Kidney disease	CMP/eGFR and urine albumin per PCP/nephrology; avoid NSAIDs.
Cancer screening	Colonoscopy history unavailable in scanned packet; PCP to verify. Dermatology skin exam yearly due immunosuppression.
Bone health	Calcium/vitamin D; consider DEXA because of steroid exposure and age.
Dental	Routine dental care after transplant clearance; antibiotic prophylaxis decisions through transplant team due allergies.

Advance care planning

Item	Documented status
Code status during transplant admission	Full code.
Health care proxy	Spouse Maria Alvarez listed as surrogate decision-maker in discharge paperwork.
Advance directive	Patient reports document exists at home; not present in scanned records. PCP requested copy.
Goals discussed	Patient prioritizes independent mobility, family time, and avoiding rehospitalization when safe outpatient management is possible.

Sample Coding and Billing Reference Snapshot - Not a Claim

Coding caution

This page is for test data and documentation parsing only. It is not a coding recommendation, claim, payer instruction, or billable record. Codes below are plausible examples that correspond to the synthetic diagnoses and therapy services in this packet.

Potential ICD-10-CM problem-code examples in this synthetic chart

Code	Description / documentation source
Z94.1	Heart transplant status - transplant discharge summary and transplant clinic notes.
D84.821	Immunodeficiency due to drugs - tacrolimus/mycophenolate/prednisone therapy.
Z79.899	Other long-term current drug therapy - transplant immunosuppression and prophylaxis monitoring.
I10	Essential hypertension - PCP follow-up with medication adjustment.
E11.65	Type 2 diabetes mellitus with hyperglycemia - A1c 7.6 and insulin use.
Z79.4	Long-term current use of insulin - active medication list.
N18.31	Chronic kidney disease stage 3a - eGFR 53.
I69.354	Hemiplegia/hemiparesis following cerebral infarction affecting left non-dominant side - therapy and neurology notes.
R26.81	Unsteadiness on feet - PT evaluation and progress note.
M62.81	Muscle weakness - PT/OT findings.
R53.81	Physical deconditioning - post-transplant rehab plan.
Z88.0	Allergy status to penicillin.
Z91.030	Bee allergy status.
Z91.040	Latex allergy status.

Potential CPT/service examples represented in therapy notes

Code	Service represented in notes
97110	Therapeutic exercise - strength/endurance training documented in PT/OT.
97112	Neuromuscular re-education - balance, left attention, coordination.
97116	Gait training - cane sequencing, turns, left foot clearance.
97530	Therapeutic activities - functional transfers, carrying, reaching, kitchen tasks.
97535	Self-care/home management - medication checklist, shower safety, ADL strategies.

Scanned Source Inventory and Record Release Notes

Source document group	Synthetic source system	Status in this packet
Demographics and safety banner	Primary care EHR	Included; allergies prominently listed.
Transplant hospitalization discharge summary	Transplant center hospital EHR	Included; 02/03/2026 - 03/02/2026 stay.
Operative and anesthesia records	Operating room source system	Included; OHT operative report and anesthesia/perfusion summary.
ICU and step-down progress notes	Hospital EHR inpatient notes	Included; representative notes from POD0, POD1, POD4, and step-down education.
CVA acute care records	External acute hospital	Included; discharge summary, neurology progress, therapy/swallow consults, stroke imaging.
Recent lab interface	Laboratory information system	Included; CBC, CMP, tacrolimus, endocrine, lipid, viral surveillance.
Diagnostic imaging	Radiology/cardiology systems	Included; brain imaging, echo, CXR, CT chest, EKG, biopsy pathology.
Outpatient rehabilitation	Rehab documentation system	Included; PT/OT evaluations, daily notes, progress note, plan of care.
Care coordination	Primary care and home health notes	Included; DME, home health, social support, preventive care.

Known synthetic chart gaps intentionally represented

Gap	Reason / realistic handling
Colonoscopy date unavailable	Common in scanned records; PCP should verify historical preventive care.
Actual advance directive not attached	Patient reports it exists at home; copy requested.
Complete inpatient medication administration record omitted	Packet includes discharge and current med reconciliation but not every inpatient MAR entry.
Full donor record omitted	Synthetic transplant clinic includes only recipient-relevant donor serostatus and surveillance.
Original 2025 inpatient rehab stay omitted	Acute stroke summary and current outpatient rehab notes provide functional continuity.

Upload test note

For OmniScribe testing, this packet should allow the application to extract allergies, active diagnoses, complex medication lists, transplant history, CVA history, recent labs, imaging, and rehab functional status.

Synthetic Scan Certification and Closing Page

Certification statement

I certify that this PDF is a synthetic demonstration record packet created for software testing. It is formatted to resemble a medical-record release packet containing scanned hospital, clinic, lab, imaging, and rehabilitation records. It does not identify a real person and is not a valid medical, legal, billing, or treatment record.

Packet name	OmniScribe_John_Alvarez_COMPREHENSIVE_SYNTHETIC_Medical_Record_Packet.pdf
Synthetic patient	John Alvarez, MRN 14332, DOB 03/14/1956
Generated for	OmniScribe patient-record upload and extraction testing
Record period represented	04/18/2025 through 05/29/2026
Key data intentionally included	Hypertension; penicillin/bee sting/latex allergies; 2025 CVA hospitalization; 2026 heart transplant with ICU/acute stay; recent labs/imaging; PCP notes; PT/OT documentation.
Prepared by	AI-generated synthetic record packet for Gil - no real PHI
Date	05/29/2026

Synthetic records clerk signature	/s/ Demo Records Clerk
Quality check	Pages numbered; synthetic watermark applied; allergy banner included; clinical sections indexed.
Release limitation	Do not release externally except as test data.

End of synthetic medical record packet